RF3237 PIL PHYSIOTENS 0.3MG XX
SAP code: 0000000 Laetus Code: 000
flat delivered leaflet 158 x 315 mm RECTO
Colour(s): PROCESS BLACK C
Replaces: RF1179 SPECIFICATION SOLID 117229
RF3237 Abbott Healthcare CCP | PPD (MS) 05-MAY-2010 (edit.2)

Physiotens® 0.3 mg

film-coated tablets
0.3 mg moxonidine

Abbott

Read this entire leaflet carefully before you start taking this medicine.
Keep this leaflet. You may need to read it again. If you have further questions, please ask your doctor or pharmacist. This medicine has been prescribed for you personally and you should not pass it on to others. It may harm them, even if their symptoms are the same as yours.

Physiotens 0.3 mg is a round, convex, pale red, film-coated tablet with a stamp "0.3" on one face, for oral administration (to be taken by mouth) containing 0.3 mg moxonidine.

Excipients (non-medicinal ingredients): Lactose monohydrate, povidone K25, crospovidone, magnesium stearate, hypromellose, ethylcellulose, macrogol 6000, talc, red ferric oxide (E 172), titanium dioxide (E 171).

Indications
Physiotens is indicated for the treatment of hypertension (high blood pressure).

Dosage and administration
Always take Physiotens exactly as your doctor has prescribed. If you have any questions, you should check with your doctor or pharmacist.

Furthermore, if you forget to take your tablet(s), do not take a double dose to compensate for it. If you require further information, please ask your doctor or pharmacist for advice.

The usual starting dose of Physiotens is 0.2 mg daily. The maximum daily dose is 0.6 mg, which should be taken as two divided doses. You should never take more than 0.4 mg at a time. The daily dose and frequency of administration may be adjusted by the treating physician according to your response.

Physiotens can be taken with or without food.

If you suffer from moderate to severe renal impairment (kidney disease) your doctor will prescribe a starting dose of 0.2 mg daily. If necessary and well tolerated your physician may then decide to increase the dose to 0.4 mg daily.

If you are undergoing hemodialysis (mechanical cleaning of the blood) the starting dose is 0.2 mg daily. If necessary and well tolerated your physician may decide to increase the dose to 0.4 mg daily.

Due to lack of data on safety and efficacy, Physiotens is not recommended for use in children and adolescents under the age of 18 years.

Contraindications
Do not take Physiotens if you suffer from one of the following conditions:
- hypersensitivity to moxonidine or to any of the excipients,
- sick sinus syndrome (heart arrhythmia due to sinus node disease)
- bradycardia (resting heart rate below 50 beats per minute).

Warnings and special precautions for use
If you are taking a β-blocker concurrently with Physiotens and your doctor decides to discontinue both treatments, the β-blocker should be discontinued first, and Physiotens only thereafter.

Treatment with Physiotens should not be discontinued abruptly. A gradual decrease in dose is recommended before complete discontinuation. Your doctor will tell you how to do this.

Patients with an intolerance to galactose (e.g. Lapp lactase deficiency or glucose-galactose malabsorption) should not take this medicine.

This product contains lactose. If you have been told by your doctor that you have an intolerance to some sugars, contact your doctor before taking this medicinal product.

Interactions with other medications
Physiotens has been safely administered with other antihypertensive drugs (drugs that lower blood pressure) such as thiazide diuretics and calcium channel blockers. Joint administration of Physiotens with these and/or other antihypertensive agents results in an additive effect (the total effect of the medication is not higher than the sum effect of each individual medication).

In healthy volunteers no pharmacokinetic interactions (alterations in absorption, action, metabolism or elimination) have been observed between Physiotens and hydrochlorothiazide, glibenclamide (glyburide), or digoxin.

Since tricyclic antidepressants may reduce the effectiveness of centrally acting antihypertensive agents (such as Physiotens), it is not recommended that tricyclic antidepressants be jointly administered with this product.

No pharmacodynamic interaction has been demonstrated with moclobemide (a medicine commonly used to treat depression and social anxiety).

Physiotens may enhance the sedative effect of benzodiazepines (sedatives) when administered concomitantly. Furthermore, Physiotens moderately enhances the impaired performance in cognitive functions in patients taking lorazepam (a benzodiazepine class sedative). If you are taking Physiotens at the same time as a benzodiazepine class sedative you should be aware of the possibility of increased drowsiness. Please take special precautions with regard to your daily activities (see also "effects on the ability to drive and use machines").

Please tell your doctor or pharmacist if you are taking or have recently taken any other medicines including medicines obtained without a prescription.

Pregnancy and lactation
No clinical data on exposed pregnancies are available for Physiotens. Animal studies did not reveal any directly or indirectly harmful effects with respect to pregnancy, embryonal/foetal development, delivery or postnatal development of the child. However, caution should be exercised when prescribing this drug to pregnant women.

Ask your doctor or pharmacist for advice before taking any medicine during pregnancy.

Moxonidine, the active ingredient of Physiotens, is excreted into breast milk. Therefore, you and your doctor should decide whether you should continue either breastfeeding or taking Physiotens.

Effects on ability to drive and use machines
No studies on the effects of Physiotens on the ability to drive and/or use machinery have been performed. However, since somnolence (sleepiness) and dizziness have been reported, this should be borne in mind when performing these tasks.

Undesirable effects
Like all medicines, Physiotens may have side effects. If you notice any side effects not mentioned in this leaflet, or if any of the side effects get serious, please inform your doctor or pharmacist.

The frequencies of study related side effects are ranked according to the following:
Common: Between 1 and 10 cases in 100 treated patients
Uncommon: Less than one case in 100 treated patients
Rare: Less than one case in 1000 treated patients
Very Rare: Less than one case in 10 000 treated patients

The most frequent side effects reported by those taking Physiotens include dry mouth, headache, dizziness, asthenia (general weakness) and somnolence (sleepiness). These symptoms often decrease after the first few weeks of treatment.

Undesirable Effects by System Organ Class:
Nervous system disorders:
Common: Headache, dizziness, somnolence (sleepiness)
Uncommon: Insomnia

Vascular disorders
Rare: Hypotension (low blood pressure), orthostatic hypotension

Gastrointestinal disorders
Common: Dry mouth
Uncommon: Nausea

Skin and subcutaneous tissue disorders
Uncommon: Rash, pruritus (itchiness)
Very rare: Angioedema (sudden swelling of the face, neck or extremities)

General disorders and administration site reactions
Common: Asthenia (general weakness)

Overdose

Symptoms of overdose

Very few cases of overdose have been reported. However, in one such report, a dose of 19.6 mg was ingested acutely without fatality. Signs and symptoms reported included: Headache, sedation, somnolence (sleepiness), hypotension (low blood pressure), dizziness, asthenia (general weakness), bradycardia (slow resting heart rate), dry mouth, vomiting, fatigue and upper abdominal pain (stomach pain).

In addition, based on a few high dose studies in animals, transient hypertension (high blood pressure), tachycardia (elevated heart rate), and hyperglycaemia (elevated blood sugar) may also occur.

Treatment of overdose

No specific antidote is known. In case of hypotension, circulatory support such as fluids and dopamine administration may be considered. Bradycardia may be treated with atropine. α-Receptor antagonists may diminish or abolish the paradoxical hypertensive effects of a moxonidine overdose.

Pharmacodynamics

Pharmacotherapeutic group: Imidazoline receptor agonists, moxonidine.

In different animal models, moxonidine has been shown to be a potent antihypertensive agent. Available experimental data suggests that the site of the antihypertensive action of moxonidine is the central nervous system (CNS). Within the brainstem, moxonidine has been shown to selectively stimulate imidazoline receptors. These imidazoline-sensitive receptors are concentrated in the rostral ventrolateral medulla, an area critical to the central control of the peripheral sympathetic nervous system. Stimulation of the imidazoline receptors appears to reduce sympathetic activity and lower blood pressure.

Moxonidine distinguishes itself from other sympatholytic antihypertensives by exhibiting only low affinity for known α_2-adrenoceptors, as compared to imidazoline receptors. This low affinity to α_2-adrenoceptors may account for a low incidence of sedation and dry mouth with moxonidine.

In humans, moxonidine leads to a reduction of systemic vascular resistance and consequently arterial blood pressure. The antihypertensive effect of moxonidine has been demonstrated in double-blind, placebo controlled, randomized studies.

In a therapeutic trial of two months' duration, moxonidine improved the insulin sensitivity index by 21% (decreased symptoms related to insulin resistant diabetes) in comparison to placebo in obese and insulin resistant patients with moderate hypertension.

Pharmacokinetics

Absorption:

After oral administration of Physiotens, the moxonidine component is rapidly (the time to maximum plasma concentration is around 1 h) and almost completely absorbed from the upper gastrointestinal tract. The absolute bioavailability (amount of drug that reaches the blood stream unchanged) is about 88%, indicating no significant first-pass (liver) metabolism. Food intake has no influence on the pharmacokinetics of moxonidine.

Distribution:

Plasma protein binding, as determined in vitro, was about 7.2%.

Biotransformation:

In pooled human plasma samples, only dehydrogenated moxonidine was identified. The pharmacodynamic activity of dehydrogenated moxonidine is about 1/10 compared to moxonidine.

Elimination:

Over a 24-hour period, 78% of the total dose was excreted in urine as parent moxonidine and 13% of the dose was excreted as dehydrogenated moxonidine. Other minor metabolites in urine accounted for approximately 8% of the dose. Less than 1% is eliminated via the faeces. The elimination half lives of moxonidine and its metabolite are approximately 2.5 h and 5 h, respectively.

<u>Pharmacokinetics in Hypertensive Patients:</u>

In hypertensive patients, no relevant pharmacokinetic changes were observed compared to healthy volunteers.

<u>Pharmacokinetics in the Elderly:</u>

Age-related changes in pharmacokinetics have been observed and are most likely due to a reduced metabolic activity and/or slightly higher bioavailability (better absorption into the blood stream) in the elderly. However, these pharmacokinetic differences are not considered to be clinically relevant.

<u>Pharmacokinetics in Children:</u>

As Physiotens is not recommended for use in children, no pharmacokinetic studies have been performed in this subpopulation.

<u>Pharmacokinetics in Renal Impairment:</u> Elimination of moxonidine is signifcantlycorrelatedwith creatinine clearance. In patients with moderaterenal impairment (GFR 30–60 ml/min) steady-state plasma concentrations and terminal half-life are approximately 2 fold and 1.5 fold higher, respec tively, compared to hypertensive patients with normal renal function (GFR > 90 ml/min). In patients with severe renal impairment (GFR < 30 ml/min) steady-state plasma concentrations and terminal half-life are approximately 3-fold higher. No unexpected drug accumulation after multiple dosing was observed in these patients. In end-stage renal patients (GFR< 10 ml/min) undergoing hemodialysis plasma con-centrations and terminal half-lives are 6 fold and 4 fold higher, respectively. In all groups maximum mox-onidine plasma concentrations are only 1.5 -2 fold higher.

In patients with renal impairment the dosage and frequency of dosage should be adjusted according to the individual's requirements.

Moxonidine is eliminated to a small extent by hemodialysis.

Incompatibilities

Not applicable.

Shelf life and storage conditions

3 years

Do not store above 30°C

Store in the original package.

Do not use the medicine after the expiry date stated on the carton.

Keep this medicine out of the reach and sight of children.

Pack sizes

Physiotens comes in packages containing 7, 10, 14, 20, 28, 30, 50, 56, 60, 84, 90, 96, 98, 100, 280, 400, 500 or 600 fi lm-coated tablets (not all pack sizes may be marketed).

The blister packs (bubble cards) are made of PVC/PVDC/Al.

Further information

Medicines should not be disposed of via wastewater or household waste. Ask your pharmacist how to dispose of medicines no longer required. These measures will help to protect the environment.

The information in this leafl et is limited. For further information, please contact your doctor or pharmacist.

Date of information

August 2023

Manufactured by:

Mylan Laboratories SAS
Route de Belleville, Lieu-dit-Maillard
01400 Chatillon-sur-Chalaronne, France

For:

Abbott Products GmbH,
Germany

laetus code > >